

The role of partnership functioning and synergy in achieving sustainability of innovative programmes in community care

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What is known about this topic

- Sustainability is increasingly important in community care because the effects of innovative programmes can be maintained over a long period.
- The relationships between partnership functioning, synergy and the sustainability of innovative programmes in community care have not been elucidated to date.

What this paper adds

- Sustainability of innovations in community care is positively influenced by leadership and non-financial resources.
- Partnership synergy acts as a mediator for partnership functioning and significantly affects sustainability of innovations in community care.
- Synergy between partners promotes sustainability of innovative programmes in community care.

Abstract

This cross-sectional study (conducted in April–May 2011) explored associations between partnership functioning synergy and sustainability of innovative programmes in community care. The study sample consisted of 106 professionals (of 244 individuals contacted) participating in 21 partnerships that implemented different innovative community care programmes in Rotterdam, The Netherlands. Partnership functioning was evaluated by assessing leadership, resources administration and efficiency. Synergy was considered the proximal outcome of partnership functioning, which, in turn, influenced the achievement of programme sustainability. On a 5-point scale of increasing sustainability, mean sustainability scores ranged from 1.9 to 4.9. The results of the regression analysis demonstrated that sustainability was positively influenced by leadership (standardised regression coefficient $\beta = 0.32$; $P < 0.001$) and non-financial resources ($\beta = 0.25$; $P = 0.008$). No significant relationship was found between administration or efficiency and programme sustainability. Partnership synergy acted as a mediator for partnership functioning and significantly affected sustainability ($\beta = 0.39$; $P < 0.001$). These findings suggest that the sustainability of innovative programmes in community care is achieved more readily when synergy is created between partners. Synergy was more likely to emerge with boundary-spanning leaders, who understood and appreciated partners' different perspectives, and could bridge their diverse cultures and were comfortable sharing ideas, resources and power. In addition, the acknowledgement of and ability to use members' resources were found to be valuable in engaging partners' involvement and achieving synergy in community care partnerships.

Keywords: community care, innovation, quality, sustainability, synergy

Introduction

In recent years, considerable resources have been devoted to the implementation and validation of innovative programmes in community care. These programmes have focused primarily on successful implementation, without creating assurances of the 'life of innovations'

after extramural funding (funding from outside the organisation that is used to support a programme or project) has ended (Goodman *et al.* 1993, Sheddie-Rizkallah & Bone 1998, Akerlund 2000, Vaughn *et al.* 2000). Given this problem, there is a general consensus that sustainability guidance is essential (Goodman & Steckler 1989, Backer 2000). Sustainability is increasingly important to

the funders and implementers of health-related demonstration programmes and innovations. The concept of sustainability refers to the continuation of programmes (Shediec-Rizkallah & Bone 1998), implying that new working methods and performance levels persist for an appropriate period of time in a given setting (Buchanan *et al.* 2005). It can then be seen as a dynamic process in which actors in a targeted work practice develop and/or adapt the organisational routines to a new work method (Slaghuis *et al.* 2011). Sustainability has been advanced as a global term to depict the continuation process, and encompasses a diversity of forms that this process may take (Shediec-Rizkallah & Bone 1998); however, no emphasis has been placed on measuring the extent of continuation.

Sustainability is important in community care because the effects of innovative programmes can be maintained over a long period (Puska *et al.* 1996, Manfredi *et al.* 2001), allowing for the study of these long-term effects (Altman 1995). For example, many community care programmes aim to increase health or well-being, and must usually survive for extended periods of time to achieve such changes (Steckler & Goodman 1989). Changes in health and well-being outcomes in most community health areas may not be detectable for 3–10 years after programme initiation (Roussos & Fawcett 2000). In addition, the absence of sustainability in programmes perceived as beneficial for the health and well-being of targeted populations represents an investment loss for the organisations and people involved (Yin 1979, Steckler & Goodman 1989, Goodman *et al.* 1993, Rissel *et al.* 1995, O'Loughlin *et al.* 1998, Shediec-Rizkallah & Bone 1998). Furthermore, the discontinuation of community programmes brings disillusion to participants, and therefore poses obstacles to subsequent community mobilisation (Goodman *et al.* 1993, Shediec-Rizkallah & Bone 1998).

A literature review focusing on the sustainability of health-related innovations in community care showed the importance of leadership. Leaders have often enthusiastically advocated for a programme's needs, particularly to help secure resources for its continuation (Scheirer 2005). More than three-fourths of studies examining influences on sustainability have cited the importance of a champion – a person whose strategic

placement within an organisation allows him/her to advocate effectively for the programme. Buchanan *et al.* (2005) found that, in addition to leadership, sustainability is dependent on resources, administration, management and efficiency. Lasker *et al.* (2001) identified leadership, efficiency, administration and management and resources as the four dimensions of partnership functioning, which can result in partnership synergy. Synergy is the degree to which the partnership combines the complementary strengths, perspectives, values and resources of all partners in the search for better solutions (Gray 1989, p. 5), and is generally regarded as the product of a partnership (Lasker & Weiss 2003). The synergy that a partnership can achieve is more than simply an exchange of resources among its partners. Theoretically, when partners effectively merge their perspectives, knowledge, and skills to create synergy, they create something new and valuable – a whole that is greater than the sum of its parts. A community care partnership is able to create synergy by combining the perspectives, knowledge and skills of diverse partners in a way that enables the partnership to do the following (i) think in new and better ways about how it can achieve its goals; (ii) plan more comprehensive, integrated programmes; and (iii) strengthen its relationship with the broader community (Lasker *et al.* 2001). The identification of factors that influence the ability of a community care partnership to attain synergy can help strengthen its capacity to create a successful collaborative process (Weiss *et al.* 2002), and thereby to achieve the sustainability of an innovative programme.

The relationships between partnership functioning, synergy and the sustainability of innovative programmes in community care have not been elucidated to date. Therefore, this study explored the associations between these factors with the aim of enhancing knowledge about the means by which partnership synergy and sustainability may be achieved in these programmes (Figure 1). In the study framework, synergy was considered the proximal outcome of partnership functioning, which, in turn, influenced programme sustainability. We hypothesised that the ability to create partnership synergy would be an essential factor for the achievement of sustainability in innovative programmes in community care.

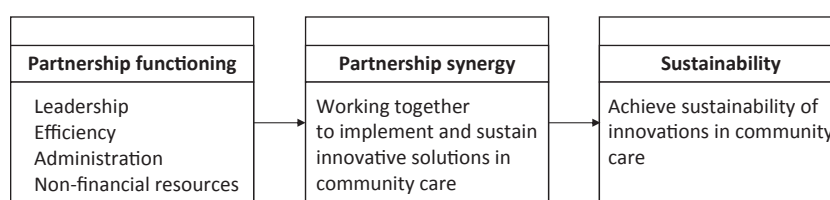


Figure 1 Theoretical model of partnership functioning, synergy and sustainability of innovative programmes in community care.

Methods

Setting

This study examined community care programmes in Rotterdam, The Netherlands. Tasks and responsibilities have been shifted increasingly from the Dutch national government to municipalities. Since January 2007, municipalities have been responsible for the execution of the 'Wet maatschappelijke ondersteuning' (a law mandating social support for communities), and have needed to find innovative ways to fulfil these new tasks. The municipality of Rotterdam has set up a fund for innovation to establish new initiatives in housing, welfare and care; this fund supported the 21 innovative projects examined in this study. To qualify for funding, the

programmes were required to be innovative and (i) enhance the self-reliance of community members and/or (ii) integrate health-care, social services and/or welfare. The 21 projects were quite diverse; for example, they were aimed at various target groups, including older persons, those with physical or mental disabilities and those with a chronic illness or life-threatening disease (see Table 1 for an overview of four programmes, full overview of 21 programmes is available from the corresponding author on request).

Procedure

All project leaders of the 21 innovative programmes in community care received an introductory letter explaining the research. They were asked to select and provide

Table 1 Overview of four innovative community care projects

Name of the project	Content of the innovative community care project
Replacement of supportive and activating coaching	The project prepares 55 individuals with a (mild) intellectual disability or psychiatric problems for the abolishment of supportive coaching and ambulatory support provided within the framework of the General Act on Special Medical Expenses (AWBZ). Two programmes aim to give clients sufficient confidence and to prepare their networks for the new situation within 1 year. One programme is aimed at stimulating the clients' networks in providing (ambulatory) assistance in living. Aiming at activation and participating, the second programme provides for assistance in (daily) activities. Clients will be made well self-sufficient in the field of social activation by offering modules and building a social network. Participation in volunteer work is one of the means to achieve this. Assistance can be fully outreaching. Supplementary facilities are available in case of lapses or impediments. The project contributes to self-sufficiency and the creation of caring networks and thus meets the goals of the Innovation Fund. Assistance activities are subsidised.
Kluscontact (chores contact)	The project involves setting up an ambulatory help team composed of volunteers, persons doing a social traineeship, and students. Collaborating with the VraagWijzer Foundation, this 20-headed volunteer team will assist isolated elderly people and chronically ill persons throughout the city. In 2 years' time some 600 persons seeking help will be attended to. The aim is to establish social contact by carrying out domestic chores. This will promote self-sufficiency and alleviate loneliness. The project also offers support to informal carers and backs up the social map around the target group. Thus, the project contributes to the Innovation Fund's goal to reinforce caring networks around vulnerable citizens. The subsidy partly covers the coordination costs.
De Nieuwe Nachtegaal	A living-, learning-, working- and health-care project will be realised in the former De Nachtegaal home for elderly people, including a neighbourhood supermarket. The residents association will take responsibility for activities in the complex, thus stimulating informal care networks. The community restaurant cum meeting centre will play a central role here. Combining learning & working programmes with day care facilities for children and for the elderly people, the neighbourhood restaurant cum grand café, and the medical care shop is an example of the added value created by establishing connections. The medical care shop in the building will be run by participants in the learning & working programmes. The shop offers domestic care on the levels of informal care, supportive coaching, preventive health-care, and activities assistance for elderly people who do not qualify for health-care support. The project plan meets the criteria of the Innovation Fund: it stimulates the informal health-care networks and links the physical and social domains. The fund subsidises the renovation work and furnishings of the community restaurant cum meeting centre.
Feel-good associations: one for elderly adults and one for children with an intellectual disability	Setting up these feel-good associations implies realising a precondition for both target groups enabling them to participate more successfully in society through sports activities. The pilot phase in part serves to identify the most desirable organisational set-up, taking into account the goals of the Social Support Act (<i>Wmo</i>) and the municipal policy on sports. Members of the target groups will be enabled to practice sports close to home and to transition to regular sports clubs. The feel-good associations will be run largely by volunteers. The VraagWijzer foundation and various institutions will participate in the project. The project qualifies for a subsidy from the <i>Wmo</i> Innovation Fund in view of the boost the feel-good associations may provide for building caring networks around vulnerable Rotterdam citizens.

contact details for preferably about 10 professionals in their partnership, which led to the inclusion of 244 potential participants. The number of professionals varied between projects depending on the size of the project (range between 1 and 25). In April–May 2011 these individuals received an email with a link to the questionnaire, which they were asked to complete online. Two weeks later, another email with a link to the questionnaire was sent to non-responders. A few weeks thereafter, the project leaders again reminded non-responders to complete the questionnaire. As this study included staff members only and not patients, we did not need approval for this study from an ethics committee. Informed consent was obtained from all participants by the project leaders. All personal identifiers were removed or disguised so the person(s) described are not identifiable and cannot be identified through the details of the story.

Measures

Sustainability was assessed with a 9-item instrument developed by Slaghuis *et al.* (2011). Examples of items are as follows: ‘the new practice is regarded as the standard way to work’ and ‘all colleagues involved in the new work practice are knowledgeable about it’. Responses are using a five-point scale, with higher scores indicating greater sustainability. The sustainability score was derived by calculating the mean of the total 9-item scores.

The Partnership Self-Assessment Tool: Short version (PSAT-S) was used to measure partnership synergy and dimensions of partnership functioning (Cramm *et al.* 2011). This instrument contains nine items that measure partnership synergy (e.g. ‘By working together, how well are the partners able to identify new and creative ways to solve problems?’) and components that measure five dimensions of partnership functioning: leadership (four items; e.g. inspiring or motivating people and taking responsibility), efficiency (three items; e.g. how well the partnership uses the partners’ financial resources), administration and management (four items; e.g. evaluating the progress and impact of the partnership and organising partnership activities, including meetings and projects), and non-financial resources [four items e.g. data and information (statistical data, information, skills, expertise)]. Responses to all items are structured using a five-point Likert scale. Butt *et al.* (2008) and Cramm *et al.* (2011) have recently confirmed the validity of the PSAT for the measurement of partnership processes. Partnership synergy and partnership functioning dimension scores were derived by calculating the sum score of responses within each concept. The partnership synergy score reflected the extent to which participants in each

partnership were accomplishing more together than they could individually.

Analyses

We used descriptive statistics to analyse the professionals’ demographic characteristics. Cronbach’s α served as a measure of homogeneity reflecting the (weighted) average correlation of items within a scale. Correlation analyses were performed to investigate relationships between partnership functioning, synergy, and the sustainability of innovative programmes in community care. Regression analyses with list-wise deletion of missing data were used to identify predictors for partnership effectiveness and synergy. In addition, Steiger’s Z tests were used to test whether or not coefficients were significantly reduced when the effects of the mediator variable (partnership synergy) was included in the model (Meng & Rosenthal 1992). We also tested for influence of partnership level (level 2) on sustainability. Results indicated that the partnership level did not influence our results ($-2 \log$ likelihood 188.14 vs. 186.19: $P = 0.161$). Therefore, we performed our analyses at individual level only. All statistical analyses were conducted with SPSS software (Release. 18.0; SPSS Inc., Chicago, IL, USA).

Results

Sample characteristics

A total of 106 respondents completed the questionnaire (43% response rate). Table 2 displays the descriptive characteristics of the sample of professionals. Of those who completed the baseline questionnaire, the majority (71.4%) were women and the mean age was 47.3 years (SD 11.1, range 21–69 years). Partnerships consisted primarily of coordinators (24.5%), managers (17.9%), project assistants (16.0%), policy officers (3.8%), communication officers (2.8%) and occupational therapists (2.8%).

Table 2 Characteristics of professionals ($n = 106$)

Characteristic	n (%) or mean (SD)
Gender	
Female	65 (71.4%)
Age (years)	47.3 (11.1)
Education	
University degree or higher	74 (69.8%)
Occupation	
Manager	19 (17.9%)
Coordinator	26 (24.5%)
Project assistant	17 (16.0%)
Policy officer	4 (3.8%)
Communication officer	3 (2.8%)
Occupational therapist	3 (2.8%)
Other	34 (32.1%)

Descriptive statistics characterising the functioning, synergy and effectiveness of the partnerships are shown in Table 3. On a 5-point scale of increasing sustainability, mean sustainability scores ranged from 1.9 to 4.9. Cronbach's α for the sustainability scale was 0.84, indicating excellent reliability. Cronbach's α values for the PSAT-S ranged from 0.93 (partnership synergy) to 0.79 (partnership non-financial resources), indicating good reliability. Table 4 shows that partnership functioning, synergy and sustainability were strongly related to one another in innovative programmes in community care (all $P < 0.001$).

Table 5 shows the direct effect of partnership functioning on programme sustainability (hierarchical regression stage 1, where the β are standardised regression coefficients). These results demonstrate that the sustainability of innovative programmes in community care was positively influenced by leadership ($\beta = 0.32$; $P < 0.001$) and non-financial resources ($\beta = 0.25$; $P = 0.008$). No significant relationship was found between efficiency or administration and programme sustainability. The results of stage 2 of the multiple regression analysis indi-

cate that, after controlling for all partnership functioning variables, partnership synergy significantly affected programme sustainability ($\beta = 0.39$; $P < 0.001$; Table 5). In addition, the strength of the previously demonstrated significant relationship between the independent variables of leadership and the outcome variable (sustainability) significantly decreased from $\beta = 0.32$ to 0.15 (Steiger's $Z = 2.26$; $P = 0.008$) when the effects of the mediator (partnership synergy) were included in the model. The same pattern emerged for the strength of the previously significant relationship between resources and sustainability, which significantly decreased from $\beta = 0.25$ to 0.14 (Steiger's $Z = 2.14$; $P = 0.005$) when partnership synergy entered the equation. This finding supports the partial mediating role of partnership synergy in the sustainability of innovative programmes in community care (Baron & Kenny 1996).

Discussion

The results of this study indicate that strong relationships connect partnership functioning, synergy and the sustainability of innovative programmes in community care, and that partnership synergy acts as a mediator between partnership functioning and sustainability. The innovative, holistic and grounded thinking of synergistic partnerships is reflected in the achievement of sustainability. Partnerships that are capable of creating synergy among members are more likely to achieve sustainability, thereby meaningfully contributing to the delivery of health and social services in the community and the improvement of population well-being.

Table 3 Descriptive properties of the measures used to examine partnership functioning, synergy and the sustainability of innovative programmes in community care ($n = 106$)

Measure	Mean (SD)	Actual range of scores	Cronbach's α
Partnership functioning			
1. Leadership	12.0 (3.1)	5.0–20.0	0.91
2. Efficiency	9.5 (1.7)	4.0–15.0	0.81
3. Administration	11.5 (3.0)	4.0–20.0	0.91
4. Non-financial resources	10.2 (2.3)	4.0–16.0	0.79
Partnership synergy			
5. Synergy	28.5 (5.2)	13.0–43.0	0.93
Programme sustainability			
6. Sustainability	3.4 (0.6)	1.9–4.9	0.84

Table 4 Pearson correlations between partnership functioning, synergy and sustainability ($n = 106$)

	1	2	3	4	5
Partnership functioning					
1. Leadership					
2. Efficiency	0.49***				
3. Administration	0.44***	0.59***			
4. Non-financial resources	0.24*	0.41***	0.54***		
Partnership synergy					
5. Synergy	0.58***	0.49***	0.44***	0.45***	
Programme sustainability					
6. Sustainability	0.50***	0.45***	0.50***	0.46***	0.63***

* $P < 0.05$; *** $P < 0.001$ (two-tailed).

Table 5 Hierarchical multiple regression analysis of partnership functioning and the mediating role of partnership synergy on sustainability ($n = 92$)

	Adjusted R^2 for equation	F-value	β (stage 1)	β (stage 2)
Partnership functioning (stage 1)	0.35	14.5***		
Leadership			0.32***	0.15
Efficiency			0.09	0.02
Administration			0.17	0.14
Non-financial resources			0.25**	0.14
Partnership synergy (stage 2)	0.45	16.1***		
Partnership synergy				0.39***

Final stage 2 model $F = 16.09$, $df = 5, 86$ and $P < 0.001$.

β , standardised regression coefficient.

** $P < 0.01$; *** $P < 0.001$.

The assessment of partnership functioning and synergy allows researchers to determine how collaboration works. In the evaluation of early efforts, programme members need to be able to document how well the partnership has achieved synergy. To strengthen the ability of partnerships to realise their full potential, participants need to know what they must do together to achieve synergy. This research showed that partners seek, through collaboration, to achieve synergy that extends beyond the mere exchange of resources. By combining partners' individual perspectives, resources and skills, a partnership creates a new and valuable whole that is greater than the sum of its individual parts (Shannon 1998, Cramm *et al.* 2011). Synergy is manifested in the thoughts and actions that result from collaboration within the partnership (Lasker *et al.* 2001).

Our study found a strong relationship between leadership and the sustainability of innovative programmes in community care. This effect was mediated by partnership synergy, which additionally indicates a strong relationship between leadership and synergy. These findings are consistent with those of other research, which has documented the importance of leadership throughout all phases of partnership development (Roussos & Fawcett 2000, Weiss *et al.* 2002, Jones & Barry 2011). High levels of synergy are associated with leadership that effectively facilitates productive interactions among partners by bridging diverse cultures, sharing power, facilitating open dialogue and revealing and challenging assumptions that limit thought and action. The identification of leadership capacities that are critical for synergy helps us to better understand the types of leaders necessary for effective planning and implementation in different situations and at various levels. Previous studies have indicated that partnerships need leaders, who are able to understand and appreciate partners' different perspectives, empower partners and perform boundary-spanning functions (Alter & Hage 1993, Wolff 2001, Weiss *et al.* 2002); these qualities may also apply to leaders of partnerships in community care. Most sectors and professions do not currently produce leaders of the type required to achieve synergy; this factor poses a key challenge for community care partnerships. In contrast, community care partnerships need boundary-spanning leaders who understand and appreciate partners' different perspectives, can bridge their diverse cultures and are comfortable sharing ideas, resources, and power (Alter & Hage 1993, McKinney *et al.* 1993). Moreover, many partnerships involve a number of people in the provision of leadership, in both formal and informal capacities (Chrislip & Larson 1994).

Our study found a strong association between partnership resources and the sustainability of innovative programmes in community care that was mediated by

partnership synergy, which suggests a strong relationship between partnership synergy and resources. Straub *et al.* (2007) also found that fostering equitable involvement by acknowledging and using members' resources, in particular their knowledge and expertise was valuable in achieving partnership synergy. Beyond basic resources, such as money, space, equipment and goods, community care partnerships need a broad array of skills and expertise to engage partners in supporting the collaboration process, and perform and coordinate the multiple components of their innovative programmes. Such skills may include communication, organisation, collaboration, information technology and evaluation. Information, which forms the basis for joint problem-solving, is also an essential resource for achieving synergy (Lasker *et al.* 2001). The types of information required by partnerships extend beyond statistical data to include the perspectives, values, and ideas of different stakeholders and community groups, as well as information about the community's assets, political environment and history. The ability of collaboration to bring together different types of information from various disciplines and the perceptions of diverse community groups is a great strength of partnership synergy.

The limitations of this study should be considered when interpreting the findings. Most importantly, the data collected were cross-sectional; as a result, causal relationships between dimensions of partnership functioning, synergy and programme sustainability could not be inferred. Until such a pathway is established, it will not be possible to confirm that synergy is a key mechanism through which community care partnerships gain an advantage over single agents in addressing health and social care issues. Furthermore, the explained variance of our final regression model was 45%. Other factors may additionally influence partnership functioning and sustainability. Jones & Barry (2011), for example found that trust affects partnership functioning and synergy, which may also affect sustainability. There may also be factors beyond the partnership that may explain sustainability. Mixed methods research (combination of qualitative and quantitative research methods) is called for to investigate other possible influences of sustainability. Finally, the sample of partnerships participating in the study was a convenience sample, rather than a random sample.

In conclusion, these exploratory results provide some support for the theoretically proposed associations between partnership functioning, synergy and the sustainability of innovative programmes in community care. Community care partnerships seem better able to achieve sustainability when synergy is created between participating professionals. In turn, partnership synergy and programme sustainability are more likely to emerge

when partnerships have boundary-spanning leaders who understand and appreciate partners' different perspectives are able to bridge their diverse cultures, and are comfortable sharing ideas, resources and power. The acknowledgement of and ability to use members' resources are also valuable in engaging partners' involvement and achieving synergy and sustainability in innovative programmes in community care.

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